Data Disaggregation for Asian American, Native Hawaiian, and Pacific Islander (AANHPI) Communities: A Comprehensive Review of Case Studies, Best Practices, Implementation Strategies, and Emerging Trends

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Executive Summary

Current data collection practices at the state and local levels group most Asian Americans into a single category. While convenient, this aggregation is dangerously misleading. It hides disparities in health, education, and economic well-being. This lack of granularity reinforces the "model minority" myth, distorts policy analysis, and directs resources away from the communities that need them most.

A review of efforts in Colorado, Chicago, Massachusetts, Oregon, and Portland reveals a pattern: disaggregation does work, but implementation consistently lags behind legislation. Key findings include:

- Data disaggregation efforts often gain momentum following a tragic incident. Localities that suffered racially motivated attacks towards AANHPI community members were able to advocate for their community's needs, including data disaggregation
- Implementation is a recurring topic across different localities. Many states pass laws but fail to fund or enforce them, leaving community organizations to "nudge" agencies with little formal power
- Data privacy has been a concern brought up by different localities. While granular data is needed for equity, the majority of the AANHPI community is foreign-born, and recent court rulings allow ICE to access sensitive data. Without proper safeguards in place, data collection runs the risk of becoming a surveillance tool

The following are a list of recommendations for localities interested in pursuing data disaggregation policy:

- **Language Disaggregation.** Collecting data on language spoken at home helps uncover hidden diversity (e.g., Cantonese vs. Mandarin needs) and improves service delivery
- **Mandate Transparency & Community Forums.** Require agencies to publish public progress reports and create formal forums for community input during implementation
- **Enforce Privacy Safeguards.** Use data masking, small cell suppression (hiding data for small groups), and multi-stage access controls (when possible) to prevent misuse by immigration enforcement
- **Build Multi-Racial Coalitions.** Align with Black, Latine, and MENA groups seeking similar disaggregation policies to share political power and resources

In conclusion, disaggregation is essential for equitable governance, but it must be paired with privacy protections and a genuine commitment to implementation.

Problem Definition

Multiple data collection practices at the state and local level across the U.S. aggregate Asian Americans into a single racial category, obscuring meaningful disparities across ethnic subgroups, distorting policy analysis, and contributing to inequitable allocation of resources. Additionally, the collection of detailed ethnic data without safeguards raises concerns about potential misuse by immigration enforcement agencies, which can discourage participation among certain communities and further compromise data accuracy and equity.

Policy Context

Local/State Level vs. Federal Level

The limitations of current federal and state data standards are deeply rooted in the longstanding challenges of capturing the nation's diversity.

Currently, the primary source of demographic data in the United States is the decennial census, conducted once every ten years. In addition, while the American Community Survey (ACS) provides more frequent estimates, its much smaller sample size may limit its ability to accurately identify trends. Despite these efforts, significant limitations remain. Broadly, these challenges fall under three categories:

1. Firstly, the census does not offer a sufficiently comprehensive range of identity options. Although the 2020 Census included its most detailed categories for AANHPI communities (including Chinese, Vietnamese, Native Hawaiian, Filipino, Korean, Samoan, Asian Indian, Japanese, Chamorro, and an “Other” category for “Other Asian” and “Other Pacific Islander"), respondents are still affected by how identities are presented on the form. While individuals can self-identify through write-in responses, groups without dedicated checkbox options or listed examples may be less visible in reported data. This creates challenges when individuals identify along multiple dimensions, such as nationality, ethnicity, or religion. For example, the 2020 Census estimated that there were approximately 70,000 Sikh Americans, while Sikh advocacy organizations suggest the true number may range from 200,000 to 500,000.[1] The discrepancy may arise because some Sikh individuals identify by national origin (e.g., Indian), while others identify by religion (e.g., Sikh). While both forms of identification are valid, this variation complicates efforts in generating precise, disaggregated data for targeted policy interventions.

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[1] Graham West, “Updated Census Figures Severely Undercount U.S. Sikhs,” Sikh Coalition, September 28, 2023, https://www.sikhcoalition.org/blog/2023/updated-census-figures-severely-undercount-u-s-sikhs/.

2. Secondly, the timing of data collection and release poses challenges for local issues. Since the census is conducted once a decade, it may fail to capture rapid demographic changes. For instance, the Arab Spring, which broke out in 2011, was a significant demographic shock to the U.S. and Europe, yet these changes may not be reflected in census data for years. Meanwhile, institutions such as healthcare systems and schools require more timely information to make responsive policy decisions—for example, targeted medical campaigns for individuals more susceptible to certain diseases or increasing funding for school counselors serving a particular ethnic background. More localized and frequent data collection efforts can address this gap by providing policymakers with up-to-date actionable insights.
3. Finally, the Census Bureau's group structure makes it difficult for transnational identities to be accurately captured. For individuals with transnational backgrounds (e.g., a Sino-Vietnamese person or a Nepali-speaking Bhutanese individual), the census may only record a portion of their identity, providing an incomplete picture that is of little use to service providers. For instance, in a community of Sino-Vietnamese residents who all identify as "Vietnamese" on the census, a service provider might reasonably offer language assistance in Vietnamese when the community's actual need is for Cantonese.

All in all, the census serves a different function than what is required at the state and local level for data disaggregation. While it provides a reliable framework for collecting and analyzing data at a national scale, its utility begins to falter when applied at the state and local levels. It is neither practical nor efficient for the census to capture every possible ethnicity in detail, as doing so would be costly, time-consuming, and difficult to implement. Instead, it is more effective to maintain the census as a federal baseline while enabling state and local institutions to conduct more granular data collection tailored to their communities' needs.

The United States is home to over 100 distinct ethnic groups and more than 350 spoken languages, making comprehensive data collection a complex endeavor. However, broad categories such as "Asian American" have proven to be not only inefficient but misleading. By collapsing diverse communities into an individual data point, the experiences of millions of Americans are rendered invisible, resulting in discrepancies in outcomes and obstacles to human flourishing. This lack of specificity hinders policymakers from accurately identifying which communities face specific barriers and creating tailored interventions to address them effectively. The need for detailed, disaggregated data is evident across several critical policy domains:

Healthcare

Aggregated health data that lumps diverse groups under the "Asian American" label creates a dangerous blind spot for public health agencies. When subgroups are invisible, disparities remain hidden, allowing preventable diseases to disproportionately burden specific communities without a targeted policy response. For example, while liver cancer is twice as common among Asian Americans compared to white Americans, this aggregate figure hides a far more severe crisis: Laotian Americans face a liver cancer rate more than seven times higher than that of white Americans.[2] Without disaggregated data, this population cannot be identified for screening programs, hepatitis B treatment initiatives, or culturally tailored prevention efforts. Similarly, cervical cancer rates reveal how aggregation obscures women's health emergencies. Hmong women in California suffer from cervical cancer at a rate of 36.6 per 100,000—more than three times the aggregated Asian Pacific Islander rate (11.8 per 100,000) and more than four times the rate for non-Hispanic white women (8.0 per 100,000).[3] These are not minor statistical fluctuations; they represent a failure to detect and respond to health crises at the community level. An example of how data disaggregation can lead to more effective policy interventions is the use of Vietnamese-language media campaigns to increase Pap smear uptake among Vietnamese American communities.[4] Health agencies are increasingly prioritizing data disaggregation because it is essential for guaranteeing public health. Accurate data enable resources to be directed to high-need communities, screenings to be deployed to high-risk areas, and interventions to be designed to address health outcome disparities.

Education

When aggregating data on educational outcomes for Asian Americans, demographic data is not only simplified—it actively conceals which students are struggling and why. According to the U.S. Bureau of Labor Statistics, 61% of Asian Americans aged 25 and older have at least a bachelor's degree (i.e., master's, PhD, etc.) compared to 38.6% of the U.S. population.[5] Additionally, according to a study by the Fordham Institute shows

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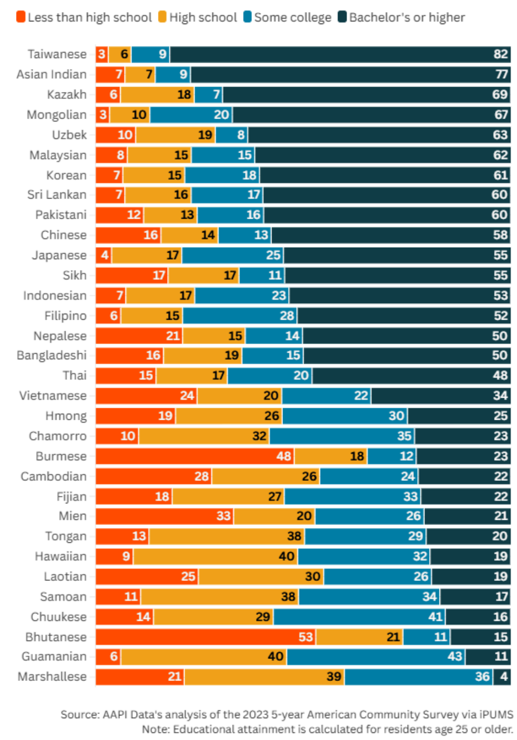
[2] Usha Lee McFarling, "Invisible in the Data: Broad 'Asian American' Category Obscures Health Disparities," STAT, November 21, 2023, https://www.statnews.com/2023/11/21/asian-american-health-disparities-obscured/

[3] Dao Moua Fang et al., "Factors Associated with PAP Testing among Hmong Women," Journal of health care for the poor and underserved, August 2010, https://pmc.ncbi.nlm.nih.gov/articles/PMC4280850/#:~:text=The%20Californian%20Hmong%20cervical%20cancer,rate%20(8.0%20per%20100%2C000).

[4] Victoria M Taylor et al., "Cervical Cancer Control Research in Vietnamese American Communities," Cancer epidemiology, biomarkers & prevention : a publication of the American Association for Cancer Research, cosponsored by the American Society of Preventive Oncology, November 17, 2008, https://pubmed.ncbi.nlm.nih.gov/18990732/.

[5] "U.S. Labor Force Characteristics of Asians, Native Hawaiians, and Other Pacific Islanders," U.S. Bureau of Labor Statistics, accessed April 2, 2026,

AAPI 8th-grade students have been rising in academic metrics over the past few decades, particularly in reading comprehension and mathematics.[6] While the AANHPI category produces high averages for college attendance, the broad category masks profound disparities in who completes high school, who accesses higher education, and who is left behind. Nationally, over 75% of Indian Americans and Taiwanese Americans hold a bachelor's degree or higher, compared to just 23% of Burmese Americans.[7] At the opposite end of the spectrum, nearly 53% of Bhutanese Americans and 48% of Burmese Americans lack a high school diploma, which are some of the lowest rates across the country.[8]



Educational Attainment by Detailed Origin for AANHPIs

The common thread across these communities is that their challenges are rendered invisible by aggregated data. Without disaggregation, policymakers cannot identify which subgroups face high dropout rates, which students lack access to academic resources, and which communities need targeted investments in English language instruction or culturally competent counseling. States such as California, Minnesota, and Washington have begun requiring disaggregated data collection in education settings, and the results have helped identify meaningful trends. In Minnesota, for example, disaggregating data by ethnicity revealed that Hmong students were less likely to be proficient on the Minnesota Comprehensive Assessments (MCA) for reading than their Chinese, Latine, Black, and White peers.[9] Disaggregation also allows English as a second language (ESL) programs to tailor instruction to students' specific linguistic backgrounds. Rather than treating all ESL learners as a monolith, disaggregated data can distinguish students by

https://blsmon1.bls.gov/blog/2024/u-s-labor-force-characteristics-of-asians-native-hawaiians-and-other-pacific-islanders.htm.

[6] Excellence gaps by race and socioeconomic status, accessed April 2, 2026, https://fordhaminstitute.org/national/research/excellence-gaps-race-and-socioeconomic-status.

[7] Karthick Ramakrishnan, Connie Tan, Akil Vohra, Clarielisa Ocampo, Jamie Noh, and Nadia Almasalkhi. “By the Numbers: Education. AAPI Data Guide on Timely Policy Issues.” Berkeley, CA: AAPI Data, 2025.

[8] Karthick Ramakrishnan

[9] Erin Hinrichs, “More Detailed Student Data Seen as Key to Narrowing the Achievement Gap,” MinnPost, February 29, 2016, https://www.minnpost.com/education/2016/02/more-detailed-student-data-seen-key-narrowing-achievement-gap/?share=custom-1557446851.

their native language and ethnicity, helping instructors choose appropriate teaching strategies. Without granularity, ESL programs risk applying a uniform curriculum that does not account for the diversity of their students.

Socioeconomic Conditions

The "model minority" myth portrays Asian Americans as a monolithic success story, citing high median incomes and educational attainment. However, this narrative collapses when data are disaggregated. In reality, Asian Americans experience the most severe income and wealth inequality of any racial group in the U.S. Nationally, one in ten Asian Americans lives below the poverty line, but this average conceals rates for specific subgroups: Burmese Americans face the highest poverty rate at 19%, followed by Hmong Americans at 17% and Mongolian Americans at 16%.[10] In New York City, the disparities are even more pronounced: approximately 32% of Bangladeshi immigrants and 29% of Pakistani immigrants live in poverty, while only 12% of Filipino immigrants live in poverty.[11] Even within Chinese American communities, Chinatown remains Los Angeles' third poorest neighborhood, with over 40% of households earning under $20,000 annually.[12] These economic disparities are not random; they are deeply structured by migration history, refugee status, and language access. Many Southeast Asian communities arrived as refugees from U.S.-involved conflicts in Vietnam, Cambodia, and Laos, while more recent arrivals continue fleeing instability in Burma/Myanmar and Bhutan. Unlike immigrants who arrive for employment or education, refugees often escape with few resources, limited formal education, and significant trauma—factors that shape intergenerational economic outcomes. Linguistic isolation also limits economic mobility. Approximately 30% of Asian Americans and 12% of Pacific Islanders have limited English proficiency (LEP).[13] This language gap affects employment, educational advancement, and access to critical benefits. For instance, when applying to government health insurance programs such as Medicare, the lack of adequate language assistance can result in enrollment challenges.[14] A study in Illinois

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[10] Kim, Juliana. "1 in 10 Asian Americans Live in Poverty. Their Experiences Vary Widely, Research Says." opb, March 27, 2024. https://www.opb.org/article/2024/03/27/1-in-10-asian-americans-live-in-poverty-their-experiences-vary-widely-research-says/.

[11] Kim, "1 in 10 Asian Americans Live in Poverty."

[12] Jian Zhao, "The Loss of Chinatown's Local Businesses Is Creating a Health Desert," USC Center for Health Journalism, October 28, 2025, https://centerforhealthjournalism.org/our-work/reporting/loss-chinatowns-local-businesses-creating-health-desert.

[13] Adewale A. Maye and Stevie Marvin, States must safeguard language access for AAPI communities as Trump undermines federal protections, May 22, 2025, https://www.epi.org/blog/states-must-safeguard-language-access-for-aapi-communities-as-trump-undermines-federal-protections/.

[14] Milkie Vu et al., "Low-Income Asian Americans: High Levels of Food Insecurity and Low Participation in the Calfresh Nutrition Program," Health affairs (Project Hope), September 20, 2023, https://pmc.ncbi.nlm.nih.gov/articles/PMC11184507/.

discovered that individuals with LEP were 5 times more likely to become disenrolled than their English-speaking counterparts.[15] Despite the need for support for AANHPI communities, reports such as the Colorado Lotus Project state that “for every $100 awarded by foundations for work in the U.S., only 20 cents are designated for AANHPI communities,” largely due to the “model minority” myth.[16] Addressing these disparities requires information on which communities are struggling and where resources must be targeted. States like California, Minnesota, and Washington have begun requiring disaggregated data collection, and community-based organizations are using this information to design culturally competent services, from small business development programs to language support services. Without such data, the "model minority" myth will continue to mask the needs of the most vulnerable Asian American communities, resulting in individuals' inability to access safety net programs or identify relevant outreach programs due to language barriers.

Case Studies

The next portion of this memo will discuss multiple case studies of recent data disaggregation efforts over the past six years. The following states and cities have advanced these efforts in their own unique ways. For instance, some have passed data disaggregation laws but are lagging in implementation (e.g., Massachusetts), while others lack official legislation but have funded projects to advance it (e.g., Colorado). Some locales are actively engaged in data disaggregation efforts (e.g., Chicago), and some are undertaking unique efforts, including language collection (e.g., Portland). The following is a summary of some of the locales I have interviewed and studied throughout my capstone research. The complete list will be sent over as a separate spreadsheet.

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[15] Milkie Vu et al

[16] “Colorado Lotus Project,” Colorado Lotus Project | Colorado Health Institute, May 2024, https://www.coloradohealthinstitute.org/programs/colorado-lotus-project.

Colorado

As of 2024, more than 320,000 individuals in Colorado identify as AANHPI.[17] While these communities have a long history in the state—including the presence of a once-thriving Chinatown in the late 19th century—systemic racism and violence, including an anti-Chinese race riot in 1880 and subsequent urban renewal efforts, led to the destruction and displacement of these early communities.[18] Despite this violent erasure, a significant AANHPI population now calls Colorado home, supported by several dedicated advocacy organizations. However, a persistent issue has hindered effective service delivery: many service providers lack clarity on where AANHPI communities are located and what their specific needs are.[19] This gap is largely due to the absence of disaggregated data, which obscures important differences across diverse ethnic subgroups. Colorado currently does not have a formal statewide policy mandating data disaggregation for AANHPI populations. In the absence of such policy, community organizations have taken the lead. The Lotus Project emerged as a collaborative initiative between the Colorado AAPI Circle and the Colorado Health Institute (CHI).[20] The Colorado AAPI Circle, a philanthropic fund dedicated to reinvesting in AANHPI communities, provided financial support and community leadership.[21] CHI, a nonprofit public health institute, contributed technical expertise in data analysis and access to critical datasets.[22]

The Lotus Project culminated in a comprehensive report published in May 2024 that sought to provide a more nuanced understanding of AANHPI communities in Colorado. The project employed both quantitative and qualitative methods, drawing from multiple data sources, including:

- U.S. Census and American Community Survey (ACS) data
- Data analyses and demographic reports published by AAPI Data
- The Colorado Health Access Survey (CHAS), administered by CHI[23]

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[17] U.S. Census Bureau. “DP05: ACS Demographic and Housing Estimates.” 2024 American Community Survey 1-Year Estimates. https://data.census.gov/table/ACSDP1Y2024.DP05?q=asian+in+colorado.
[18] History Colorado, “The Rise and Fall of Denver’s Chinatown,” published April 11, 2019, accessed April 19, 2026, https://www.historycolorado.org/story/colorado-voices/2019/04/11/rise-and-fall-denvers-chinatown.
[19] Jin A. Tsuchiya (Co-Chair and Founding Member, at The Colorado AAPI Circle) interview with the author, April 2026.
[20] Colorado Health Institute, Colorado Lotus Project: A Statewide Look at the Strengths and Barriers Facing Colorado's Asian American and Native Hawaiian and Other Pacific Islander Communities (May 2024), https://www.coloradohealthinstitute.org/sites/default/files/2024-05/Colorado%20Lotus%20Project%20Web.pdf.
[21] Lindsey Whittington (Data and Analysis Manager at Colorado Health Institute) interview with the author, March 2026
[22] Whittington, Interview.
[23] Colorado Health Institute, Colorado Lotus Project.

The inclusion of CHAS was particularly valuable, as it offered:

- State-level specificity not available in national datasets
- Insights into Colorado's unique health insurance landscape
- Recent data on healthcare access barriers and utilization patterns[24]

In addition to quantitative data, the project incorporated community interviews and translated outreach and data collection materials into 7–8 languages, with attention to dialect-specific needs.[25] This approach helped ensure broader accessibility and cultural relevance. The project was largely volunteer-driven, reflecting a significant investment of time and effort by members of the AAPI Circle and CHI.[26] It represents a notable example of community-led data infrastructure development in the absence of formal state policy.

The report highlighted substantial disparities that are obscured by aggregated data in Colorado, such as the following:

- **Income and Poverty:** While the overall poverty rate for Asian Coloradans (21%) appears slightly lower than the state average (24%), certain subgroups, such as Burmese and Bangladeshi communities, face significantly higher poverty rates of 52% and 49%, respectively.
- **Mental Health:** Aggregated data suggests relatively positive outcomes, but subgroup analysis reveals disparities—for example, 22% of Korean respondents reported poor mental health, exceeding the state average of 12.4%.
- **Language and Cultural Access Barriers (Health & Education):** Respondents highlighted systemic challenges across both healthcare and education, including language barriers, a shortage of culturally competent providers and educators, limited access to linguistically appropriate services, and a broader need for culturally responsive systems and greater diversity within the workforce.[27]

The Lotus Project was completed within an eight-month timeline, a relatively short period for a project of its scope. This created several challenges:

- **Community Reach:** Certain populations—particularly refugee communities—were difficult to engage due to language barriers, geographic isolation, and distrust of institutions.
- **Capacity Constraints:** In some regions, public health infrastructure is limited; for example, a single epidemiologist may serve an entire region.

[24] Whittington, interview.
[25] Whittington, interview.
[26] Tsuchiya, interview.
[27] Colorado Health Institute, Colorado Lotus Project.

- **Trauma and Distrust:** Many refugee populations have experienced government persecution, making participation in data collection efforts—especially those perceived as official—challenging.
- **Youth Representation:** The report included limited input from younger AANHPI individuals, indicating a gap in generational perspectives.[28]

Following its release, the Lotus Project was integrated into the Colorado Asian Culture and Education Network (CACEN) to support its continued expansion and long-term sustainability.[29] Stakeholders have expressed interest in developing future reports to track trends over time and deepen understanding of AANHPI community needs through longitudinal analysis.[30]

Despite the success of the Lotus Project, efforts to advance statewide data disaggregation policies in Colorado face political and institutional barriers. Advocates have raised concerns about potential federal opposition and broader political dynamics that may limit the feasibility of passing such legislation in the near term.[31] Nevertheless, there is clear momentum at the community level. Existing infrastructure, such as partnerships between philanthropic organizations and research institutions, positions Colorado to advance data disaggregation efforts when policy conditions become more favorable. In the interim, alternative approaches may prove effective. Nonprofit organizations, academic institutions, and community-led initiatives like the Lotus Project can continue to play a critical role in generating disaggregated data and informing service delivery. However, the ultimate end goal should be for the state itself to collect disaggregated data. Colorado already has the necessary institutions and infrastructure to conduct widespread data collection. Ideally, the state will eventually build on the foundation laid by community-led initiatives like the Lotus Project, transitioning from philanthropic efforts to a statutory mandate. In other words, disaggregation should not remain dependent on the capacity of nonprofit organizations or academic partners. Rather, it should become a routine, state-led function codified through legislation.

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[28] Whittington, interview.
[29] Tsuchiya, interview.
[30] Tsuchiya, interview.
[31] Tsuchiya, interview.

Chicago

Chicago is an instructive case because data disaggregation has been passed at the state level: Illinois passed a data disaggregation law requiring a Middle Eastern and North African (MENA) category in 2023.[32] Despite being one of the earlier adopters of data disaggregation along with being a model for other states to follow, there is still much to be done for data disaggregation within the windy city of Chicago.

Chicago has a long history as a diverse city, shaped in part by major migration waves such as the Great Migration, as well as the successive waves of European, Latine, and Asian immigration patterns. Today, the city is home to a growing Chinatown, alongside a growing MENA population. Chicago's AANHPI community is also expanding, growing at a rate of 11.7% between 2020 and 2024.[33] Chicago is the third-largest city in the U.S., and it continues to experience economic and racial inequities. Historically, the city has experienced significant segregation, with many communities facing poverty and disinvestment. These conditions underscore the need for targeted policy interventions and for more granular, disaggregated data at the local level to inform those efforts. In response to these gaps, the Chinese American Service League (CASL) launched Change InSight, a national coalition of nonprofits that utilizes community-driven data to amplify AANHPI voices. The initiative began with six partner organizations in Chicago and has expanded to approximately 30 partners across the U.S.

Given CASL's experience in data collection, it is also at the forefront of efforts to pass Ordinance O2025-0021028 through the Chicago City Council.[34] It is important to note that the ordinance has not been passed yet and can be subject to changes and provisions. The proposal aims to expand the collection and reporting of disaggregated racial and ethnic data across city agencies. This effort is being led by Alderwoman Nicole Lee in collaboration with CASL and is straightforward in that it includes the following:

Level of Disaggregation: The legislation advocates for the collection of data on AANHPI groups, including Asian Indian, Filipino, Chinese, Korean, Pakistani, Vietnamese, Japanese, Thai, Burmese, Taiwanese, Polynesian, Native Hawaiian, Micronesian, Samoan, Chamorro, Melanesian, Fijian, Guamanian, Palauan, Marshallese, and an "Other" category.[35]

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[32] Uniform Racial Classification, Illinois Public Act 103-0414 (2023). https://www.ilga.gov/legislation/publicacts/fulltext.asp?Name=103-0414.
[33] City of Chicago, Ordinance O2025-0021028, "Amendment of Municipal Code Title 2 by adding new Chapter 2-41 to require Department of Public Health to annually collect race and ethnicity data of city residents using subcategories for Asian or Pacific Islander," passed by the Chicago City Council, 2025.
[34] City of Chicago, Ordinance O2025-0021028.
[35] City of Chicago, Ordinance O2025-0021028.

Impacted Agencies: The agencies that will be required to disaggregate data on race and ethnicity are the city's Department of Public Health and the city's Department of Family and Support Services (DFSS).[36]

Data Privacy: There is a specific provision to ensure that the data is de-identified to preserve anonymity for respondents. Any requests to release the data must be subject to the city council's approval as well.[37]

Implementation: There is no clause around implementation, though this omission may have been intentional. According to representatives from CASL—who helped draft the legislation—explicit implementation language was excluded to ensure its passage.[38] Nevertheless, the ordinance does require the city to publish an annual report, which may serve as a mechanism to ensure the work gets done. The legislation appears to be in a very early stage of the legislative cycle and may be subject to amendments as it moves through committee.

Implementation Date: The current state of the legislation states that the ordinance "takes effect upon passage and approval."[39] Additionally, following the legislation's passage, the ordinance calls for a public meeting within 60 days. The purpose is to solicit advice from AANHPI-serving organizations to assist with implementation.[40] Finally, the ordinance requires the DFSS to publish an annual report, on or before July 1st of each year, on the progress made in distributing resources to AANHPI communities.

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[36] City of Chicago, Ordinance O2025-0021028.
[37] City of Chicago, Ordinance O2025-0021028.
[38] Alex Montgomery (Senior Director, Impact & Advocacy at CASL) interview with the author, February 2026.
[39] City of Chicago, Ordinance O2025-0021028.
[40] City of Chicago, Ordinance O2025-0021028.

Massachusetts

Massachusetts has a sizable AANHPI population of approximately 500,000 residents, representing about 7% of the state's total population.[41] The state of Massachusetts operates on a two-year legislative cycle, which shapes how policy proposals are introduced and advanced. Despite Massachusetts's progress in data disaggregation policy, it did not initially pass standalone legislation. In 2018, lawmakers introduced House Bill (HB) 3361, which sought to require more comprehensive disaggregation of AANHPI data at the state level. The bill faced organized opposition, with critics arguing that it could alienate AANHPI individuals and reinforce the perception of AANHPI individuals being perceived as 'perpetual foreigners.'[42] While many organizations supported the bill, it was eventually replaced by House Bill 4408, which proposed establishing a commission to study the feasibility and implications of collecting disaggregated data.[43] However, HB 4408 was ultimately killed in committee, despite recommendations for advancement with amendments.

One reason for the legislation's failure was a lack of public awareness and limited political education around the importance of data disaggregation.[44] This was evident in the size of organized opposition, including individuals who traveled from New York and New Jersey to testify against the bill.[45] Transparency challenges have also shaped the policy landscape in Massachusetts. The Ways and Means Committee, which played a key role in deliberating the legislation, is not required to take or publicly release meeting notes, limiting community visibility into the decision-making process.[46] In 2023, however, the Massachusetts state budget adopted provisions advancing data disaggregation. Despite the passage of these measures within the state's budget, implementation has faced several setbacks. The data disaggregation provisions in the state budget included the following:

Level of Disaggregation: One of the most extensive data disaggregation efforts in the country. Also includes data disaggregation for Black/African American individuals, white individuals, Hispanic/Latine individuals, etc. The data equity bill states the following:

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[41] S. Dhaurali and P. Watanabe, No Longer Invisible: 2025 Massachusetts Asian and Pacific Islander American (APIA) Community Survey Report (Boston, MA: Institute for Asian American Studies at UMass Boston and Asian American and Pacific Islanders Commission, June 3, 2025).
[42] Ryan General, "Disturbing Massachusetts Bill Pushes for Data Collection of Asian-Americans," NextShark, December 19, 2021, https://nextshark.com/disturbing-massachusetts-bill-pushes-data-collection-asian-americans.
[43] "Bill H.4408 190th (2017 - 2018)," Bill H.4408, accessed April 5, 2026, https://malegislature.gov/Bills/190/H4408#:~:text=Bill%20History%20Amendments-,Bill%20History,consumer%20impacts%20of%20electronic%20textbooks.
[44] Jaya Savita (Executive Director of API CAN) interview with the author, March 2026
[45] Savita, interview
[46] Town of Needham, MA, "What Meetings Are Covered by the Open Meeting Law?," accessed April 5, 2026, https://www.needhamma.gov/4431/Covered-Meetings.

(i) **each major Asian group**, as reported by the United States Census Bureau, including, but not limited to, Chinese, Japanese, Filipino, Korean, Vietnamese, Asian Indian, Laotian, Cambodian, Bangladeshi, Hmong, Indonesian, Malaysian, Pakistani, Sri Lankan, Taiwanese, Nepalese, Burmese, Tibetan and Thai;
(ii) **each major Pacific Islander group,** as reported by the United States Census Bureau, including, but not limited to, Native Hawaiian, Guamanian, Samoan, Fijian and Tongan;
(iii) each other Asian or Pacific Islander group not listed in clause (i) or (ii);[47]

Impacted Agencies: All state agencies that already collect race and ethnicity data are required to disaggregate data[48]

Data Privacy: The legislation contains explicit language around abiding by federal and state regulations when it comes to safeguarding data.[49] During my interview with API CAN, I was also told that advocates pushed for stronger protections, including the possibility that if the state gets subpoenaed, any shared information would not be traceable to individuals.[50]

Implementation: Despite the legislation passing in 2023, implementation has lagged. Key challenges around implementation include the following:

1. **Lack of pilot testing**: Despite initial plans, no pilot program was implemented to test data collection methods[51]
2. **Implementation delays**: State agencies have been slow to act, often citing budget constraints.[52] Agencies that are required to disaggregate data can also apply for waivers to extend their implementation deadline for prolonged periods of time.[53]
3. **Insufficient community engagement**: Community organizations have had limited input in implementation. As noted by API CAN leadership, their role has largely been to “nudge and push” policymakers, rather than actively shape the process[54]

Implementation Date: The law itself was implemented in January 2026, but implementation has been lagging due to the aforementioned concerns.

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[47] Massachusetts FY2024 Enacted Budget, Outside Section 7 (Data Equity) and Outside Section 104 (Data Equity Effective Date 2),
https://budget.digital.mass.gov/summary/fy24/outside-section/section-7-data-equity/.
[48] Mass. FY2024 Enacted Budget, Outside § 7.
[49] Mass. FY2024 Enacted Budget, Outside § 7.
[50] Savita, interview.
[51] Savita, interview.
[52] Savita, interview.
[53] Savita, interview.
[54] Savita, interview.

Oregon & Portland

This section will focus on two localities that overlap with one another because their policies are interlinked. In this section, I will be discussing the city of Portland and the state in which it lies, Oregon. The reason why I am choosing to do so is because Oregon passed protocols around data disaggregation in 2014 and has passed legislation updating their data disaggregation standards within the last six years.[55] Furthermore, Portland has more recently adopted a more holistic approach to data disaggregation by requiring additional agencies to implement disaggregated data practices, but understanding this effort requires broader context about Oregon's data disaggregation landscape.

Oregon

In 2014, the state of Oregon implemented a set of data collection standards titled Race, Ethnicity, Language, and Disability (REALD) to help track, measure, and eliminate health inequities.[56] REALD was first codified in the Oregon Administrative Rules (OAR) in 2014, directing state agencies to standardize demographic data collection.[57] Following the COVID-19 pandemic, which revealed critical data gaps in treatment, the state legislature passed HB 4212 in 2020, specifically requiring REALD data collection for COVID-19 encounters.[58] This was followed by the landmark bill HB 3159 in 2021, which mandated the creation of a permanent, secure data collection system for both REALD and the newly added SOGI (Sexual Orientation and Gender Identity) standards.[59]

To comply with HB 3159, the Oregon Health Authority (OHA) began building an integrated data repository and registry system.[60] The REALD & SOGI Repository is a centralized, operational database that links demographic data from multiple state sources—including the ONE eligibility system, vital records (OVERS), and communicable disease surveillance systems (ORPHEUS, MM)—to create more complete demographic information.[61]

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[55] Oregon Health Authority, "REALD & SOGI," accessed May 3, 2026, https://www.oregon.gov/oha/ei/pages/reald.aspx.
[56] Oregon Health Authority, "REALD & SOGI."
[57] Oregon Health Authority, "REALD & SOGI."
[58] Oregon Medical Board, "REALD Data Collecting and Reporting Requirements," accessed May 5, 2026, https://www.oregon.gov/omb/topics-of-interest/pages/reald-data-reporting.aspx.
[59] Oregon House Bill 3159, 2021 Regular Session (Or. 2021). Accessed May 2, 2026. https://olis.oregonlegislature.gov/liz/2021R1/Measures/Overview/HB3159.
[60] Oregon House Bill 3159.
[61] Oregon Health Authority, "CCO Metrics Demographic Methodology," published November 25, 2025, accessed May 6, 2026, https://www.oregon.gov/oha/hpa/analytics/pages/cco-metrics-methodology.aspx

However, the full implementation of the REALD and SOGI standards has lagged significantly. The primary reason for the delay is the development of the REALD & SOGI Registry—the separate system required for mandatory annual data submissions by providers, insurers, and CCOs.[62] This system was originally anticipated to be completed much sooner, but due to factors such as limited staffing, underfunding, and technical complexity, the go-live date has been pushed back to late 2026 or 2027.[63] Once the system is completed, entities like providers, insurers, and CCOs will be required to submit REALD and SOGI data annually, a major step toward achieving data justice.

		2014 (Baseline)	2020	2024
Race / Ethnicity	Open Text	Yes	No change	No change
	# of Identities	34	Added 5 (total 39)	Added 33 to accommodate OMB updated changes and per RAC (total 74, including 7 free text fields)
	Primary Race	Yes	Revised for clarity	No change except for minor editing
Language	Languages Preferred	Spoken, Written	Added Home Language	Added question about home language to help create skip patterns
	Interpreter Needs	Two questions – need Interpreter; need Sign Language Interpreter	Revised interpreter need questions for clarity (combining interpreter and sign language need and added xx options for types of interpreters)	Added 2 additional types of interpreters
	Other Language Access Needs	Alternative Format	Dropped alternate format question.	Added a new question about captioning and assistive device needs with 5 number of response options

Summary of Changes to REALD & SOGI Data Collection Standards[64]

General details around Oregon's data disaggregation efforts are as follows:

Level of Disaggregation:

- **Asian:** Afghan, Asian Indian, Cambodian/Khmer, Chinese, Communities of Myanmar, Filipino/a, Hmong, Indonesian, Japanese, Korean, Laotian, Pakistani, South Asian, Taiwanese, Thai, Vietnamese, and Other (a text box option is available).

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[62] Oregon Health Authority, Equity and Inclusion Division, OHA and ODHS REALD and SOGI Legislative Report 2024, June 20, 2024, https://www.oregon.gov/oha/EI/Reports/REALDSOGILegislativeReport_2024.pdf.

[63] Oregon Health Authority, REALD and SOGI Legislative Report 2024, 2024.

[64] Oregon Health Authority, REALD and SOGI Legislative Report 2024, 2024.

- **Native Hawaiian and Pacific Islander:** Marshallese, CHamoru (Chamorro), Samoan, Fijian, Tongan, communities of the Micronesian region, Native Hawaiian, and Other (a text box option is available).[65]

Impacted Agencies: Oregon Health Authority (OHA) and the Oregon Department of Human Services (DHS) must collect REALD.[66]

Data Privacy: Legislation such as HB 3159 mandates that all data collected on race and ethnicity be aggregated to ensure data privacy when published and/or shared.[67] Additionally, access to the data repository is limited to ensure that only a small number of individuals have access to secure client data.[68]

Implementation:

- 950-030-0200 (Reporting Progress on Implementation):
    - "(1) All programs of the Oregon Health Authority (OHA) that collect demographic data must report to the OHA Equity and Inclusion Division in February of each even-numbered year. Reports shall include information about each program's:
        - (a) Progress in implementing these standards.
        - (b) Challenges to full implementation of these standards.
        - (c) Plan for addressing challenges, including identifying responsible staff and timeline.
    - (2) The OHA Equity and Inclusion Division shall use data provided by the programs to create a report for the legislature as required by ORS 413.162."[69]
- With regards to the data repository, implementation is ongoing. There was a three-phase method of implementing REALD data that included the following:
    - Phase 1: Patient-Facing Survey Tool. A web-based form that allows clients to enter their REALD information to automatically report their data to the OHA.
    - Phase 2: Repository. Ingesting data from different data sources (such as ONE, COVID data, etc.) The repository is already active and data is being collected from multiple sources
    - Phase 3: Registry and Integrations. Developing the data to create a registry that will receive data from different entities and serve as the primary hub of REALD information. Intended to be implemented by 2026

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[65] Oregon Health Authority, "EID 2-2024, Chapter 950: Equity and Inclusion Division – REALD and SOGI Demographic Data Collection and Reporting," filed July 2, 2024, accessed May 6, 2026, https://www.oregon.gov/oha/EI/REALD%20Documents/EID_2-2024.pdf.
[66] Oregon Health Authority, REALD and SOGI Legislative Report 2024, 2024.
[67] HB 3159, 2021 Regular Session (Or. 2021).
[68] Aileen Alfonso Duldulao (REALD & SOGI Data Equity Measurement Methodologist at Oregon Health Authority) interview with the author, May 2026
[69] Oregon Health Authority, "950-030-0200: Reporting Progress on Implementation," in Equity and Inclusion Division – Chapter 950, Division 30: REALD and SOGI Demographic Data Collection Standards, effective July 2, 2024, accessed May 6, 2026, https://secure.sos.state.or.us/oard/viewSingleRule.action?ruleVrsnRsn=315088.

or 2027.[70]

Implementation Date: While REALD standards are set in place, the data repository has been lagging and is anticipated to be completed by late 2026 or 2027.[71]

Portland

In 2024, the City of Portland, through its Office of Equity and Human Rights, adopted Administrative Rule ADM-18.03, establishing standardized demographic data collection standards.[72] Known as REALD with Tribal Affiliation (RELDTA), the rule applies to all city staff and, by extension, to contractors and grantees when collecting demographic information from the public—including individuals accessing city services or programs, participating in community engagement, or working in partnership with the city through contracts or grants.[73] Portland's rule establishes uniform practices for data collection across several domains: race and ethnicity, language, disability, and American Indian or Alaska Native tribal affiliation.[74] The rule also mandates that data be collected and stored in a manner that ensures anonymity and privacy, with data typically collected separately from other personally identifiable information where possible.[75] Unlike Oregon's statewide REALD standards, which are tied to healthcare providers and a centralized data registry that remains under development, Portland's rule went into effect upon adoption and applies directly and immediately to all city bureaus, contractors, and grantees.[76] Implementation is monitored by the Office of Equity as part of civil rights compliance, and the standards are reviewed every three years by the Office of Equity and the City Data Governance Committee to ensure they remain up to date and consistent with best practices.[77]

What makes Oregon and Portland unique is that they also mandate the collection of language data, as well. Due to REALD/RELDTA standards, entities like Oregon Health Authority (OHA) are required to collect data on language.[78] The reason why, as mentioned on the agency's website, is due to eliminating barriers that exist in access to

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[70] Oregon Health Authority, REALD and SOGI Legislative Report 2024, 2024.
[71] Oregon Health Authority, REALD and SOGI Legislative Report 2024, 2024.

[72] City of Portland, "ADM-18.03 - Race, Ethnicity, Language, Disability, and Tribal Affiliation Demographic Data Standards," adopted May 2, 2024, accessed May 6, 2026, https://www.portland.gov/policies/administrative/civil-rights/adm-1803-race-ethnicity-language-disability-and-tribal.
[73] City of Portland, "ADM-18.03."
[74] City of Portland, "ADM-18.03."
[75] City of Portland, "ADM-18.03."
[76] City of Portland, "ADM-18.03."
[77] City of Portland, "ADM-18.03."
[78] Oregon Health Authority, "REALD & SOGI," Oregon.gov, accessed May 1, 2026, https://www.oregon.gov/oha/ei/pages/demographics.aspx.

care.[79] The collection of data can aid healthcare providers in identifying what interpreters should be present in the community, as well as acknowledging the barriers that may exist in asking for interpreters.[80]

General details around Portland's data disaggregation efforts are as follows:

Level of Disaggregation:

- **Asian:** is a demographic category that includes all individuals who identify with or descend from any of the original peoples, nationalities, or ethnic groups of East Asia, Southeast Asia, or the Indian subcontinent. Examples of these groups could include but are not limited to Asian Indian or East Indian, Bengali, Cambodian, Chinese, Filipino, Hmong, Japanese, Korean, Mien, Pakistani, Thai, Vietnamese, etc.[81]
- **Native Hawaiian or Pacific Islander:** This is a demographic category that includes all individuals who identify with or descend from any of the original peoples, nationalities, or ethnic groups of Hawaii, Guam, Samoa, or other Pacific Islanders. Examples of these groups could include, but are not limited to, Carolinian, Chamorro, Chuukese, Fijian, and Marshallese.[82]

Impacted Agencies: "This rule applies to all City of Portland staff when collecting demographic information about the public."[83]

Data Privacy: The city of Portland has multiple resolutions and policies around data privacy, such as HRAR-11.04 (Protection of Restricted and Confidential Information)[84] and Resolution 37437 (Establish Privacy and Information Protection Principles for how the city collects, uses, manages, and disposes of data and information).[85]

Implementation: The Office of Equity may develop shared definitions, policies, and practices that expand on the minimum criteria of these rules. In partnership with the City Data Governance Committee, the office will also provide ongoing implementation guidance for collecting data in culturally competent ways; protecting client privacy; adding demographic categories to improve data accuracy; and creating education and communication materials. As part of civil rights compliance monitoring, the Office of

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[79] Oregon Health Authority, "CCO Metrics Demographic Methodology."
[80] Oregon Health Authority, "CCO Metrics Demographic Methodology."
[81] City of Portland, "ADM-18.03."
[82] City of Portland, "ADM-18.03."
[83] City of Portland, "ADM-18.03."
[84] City of Portland Bureau of Human Resources. "HRAR-11.04: Protection of Restricted and Confidential Information." City of Portland Policies. Adopted April 25, 2016. Accessed May 4, 2026. https://www.portland.gov/policies/human-resources-administrative-rules/ethical-conduct/hrar-1104-protection-restricted-and
[85] City of Portland, Oregon. Efiles Record No. 13112757. Portland City Archives. Accessed May 4, 2026. https://efiles.portlandoregon.gov/Record/13112757

Equity will assess how City employees use these standards. Additionally, the office and the city data governance committee, along with bureau stakeholders, will review the standards every three years to ensure they remain current, efficient, uniform, and aligned with the best, promising, and emerging practices.[86]

Implementation Date: Enacted May 2024.[87]

[86] City of Portland, "ADM-18.03."
[87] City of Portland, "ADM-18.03."

Trends

My research has revealed the following trends:

Political Momentum in Times of Crises

Across multiple locales, data disaggregation efforts often gain traction in the aftermath of racially motivated violence targeting AANHPI communities. Former Governor Andrew Cuomo vetoed data disaggregation legislation in New York State in 2019, prior to the COVID-19 pandemic.[88] Following the COVID-19 pandemic and the rise in AANHPI hate crimes, coalition and advocacy groups renewed their efforts, and legislation was ultimately passed amid heightened awareness of anti-Asian hate incidents. Similar policy efforts have emerged following tragic events nationwide, including the Atlanta spa shootings and increased violence against AANHPI elders.

It's important to note that violence isn't needed to create this kind of legislation; instead, policymakers usually put data disaggregation on the back burner until a significant event grabs public attention and encourages advocacy. These moments of crisis create the political will necessary to advance policies that may have previously stalled. The shift underscores how external events can reshape political priorities and create openings for previously unsuccessful policy proposals.

Data Security

There is a growing emphasis on incorporating data security measures into data disaggregation efforts. While government agencies already operate under established standards for handling sensitive information, there is increasing concern about protecting individual and community privacy—particularly in relation to how data may be accessed or used by federal agencies.

These concerns have been heightened by reports of increased U.S. Immigration and Customs Enforcement (ICE) activity in places such as Minnesota, where agents were reportedly seeking information about the location of Asian communities.[89] In response, advocacy organizations have worked with local and state governments to limit the sharing of sensitive demographic data with federal authorities.[90]

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[88] Sakshi Venkatraman, “N.Y. Enacts ‘groundbreaking’ Law to Change How Asian American Populations Are Counted,” NBCNews.com, December 29, 2021, https://www.nbcnews.com/news/asian-america/new-york-state-disaggregate-data-asian-american-groups-rcna10237?trk=article-ssr-frontend-pulse_little-text-block.

[89] Edith Olmsted, “ICE Agents Reportedly Asking Minnesotans Where the Asians Are,” The New Republic, January 14, 2026, https://newrepublic.com/post/205297/ice-minnesota-racial-profiling-refugees-deportations.

[90] Whittington, interview.

A similar tension emerged in Colorado through the Lotus Project. Researchers sought to develop a mapping tool to help AANHPI philanthropic organizations identify and support communities in need. The Colorado Health Institute (CHI) created a platform that pinpointed the concentration of specific populations, allowing for more targeted resource allocation.[91] However, this effort raised concerns about potential misuse—particularly the risk that such data could be used to target vulnerable communities, including Southeast Asian populations, in immigration enforcement actions.[92] In response, CHI implemented strict access controls, requiring users to complete a detailed questionnaire and limiting who could view and retain access to the data.[93]

Similarly, Chicago has incorporated data privacy considerations into its data disaggregation efforts.[94] While privacy has always been a component of data governance, a new layer of concern has emerged regarding the potential for federal access to localized data. This has led some state and local governments to reevaluate data-sharing practices, particularly in relation to federal immigration enforcement.

More broadly, this situation reflects an evolving policy landscape in which data disaggregation efforts must balance two competing priorities: the need for granular, actionable data and the responsibility to safeguard vulnerable communities. As jurisdictions continue to refine these policies, questions around data ownership, access, and intergovernmental sharing will remain central to the debate.

Implementation Approaches: Broad vs. Targeted Agency Models

A key trend in implementation is how jurisdictions structure which agencies are responsible for collecting disaggregated data. In practice, most approaches fall into one of two models:

1. **Agencies already collecting demographic data:** Under this model, any agency that currently gathers information pertaining to race and ethnicity is required to expand its data collection to include broader categories. The advantage of this approach is its efficiency: it builds on existing infrastructure and ensures broader adoption across government. However, implementation can be uneven and slow. For example, Massachusetts allows agencies to apply for waivers to delay

[91] Whittington, interview.
[92] Whittington, interview.
[93] Whittington, interview.
[94] City of Chicago, Ordinance O2025-0021028.

compliance, which has led to postponed implementation and reduced overall impact.[95]

2. **Targeted Agencies:** Under this model, specific agencies are targeted for implementation (e.g., agencies, such as departments of health, education, and/or family services.) This model has been used in places like Chicago, where laws only apply to certain agencies instead of all of them. The rationale is to align data collection with specific policy goals. For instance, education agencies may seek more detailed data on Southeast Asian students' mental health outcomes to inform targeted interventions, such as hiring culturally competent counselors or expanding school-based mental health services.

The "targeted" approach can be more politically feasible, as it limits the scope of implementation and reduces immediate costs, making it easier to secure legislative approval. However, its narrower focus means that fewer agencies participate, potentially limiting the breadth of data available for cross-sector policymaking. Ultimately, these two models reflect a tradeoff between scale and political feasibility: broad approaches maximize coverage but may face implementation delays, while targeted approaches are easier to pass but may produce more limited datasets. A hybrid approach may offer the most sustainable path for long-term data disaggregation efforts: localities could begin by targeting a specific set of agencies, then gradually expand requirements across additional agencies as implementation capacity further develops.

Arguments Opposing Data Disaggregation and Their Counterarguments

Across multiple locales, debates around data disaggregation often center on concerns about equity, cost, and privacy. Below is a synthesis of common arguments and counter-arguments:

- **Argument #1 Against Disaggregation:** Data disaggregation is a form of "discrimination" and perpetuates the perception that Asian Americans are "foreigners." Critics also argue that it is divisive and may fragment the AANHPI community.
    - **Counterargument:** Data disaggregation does not discriminate; instead, it guarantees recognition for all voices and addresses the unique needs of various AANHPI communities.
- **Argument #2 Against Disaggregation:** Implementation is too expensive for agencies and may require costly technological upgrades.
    - **Counterargument:** Technological modernization is essential for effective governance. Data disaggregation can act as a catalyst for agencies to upgrade systems, ultimately improving efficiency and service delivery.
- **Argument #3 Against Disaggregation:** Constituents and legislators have raised concerns about data privacy and potential misuse.
    - **Counterargument:** Many jurisdictions have incorporated explicit

[95] Savita, interview

safeguards in their ordinances to protect privacy, including secure storage, restricted access, and clear protocols for data use.

Implementation Trends

Implementation varies more widely across locales than the passage of data disaggregation policies themselves, driven by several key factors. Some locales have adopted data disaggregation through legislative means, passing local or state legislation. Others have updated agency policies (e.g., Oregon's Health Authority) or embedded data disaggregation into budget measures (e.g., Massachusetts). A third pathway includes executive orders or administrative directives, though these are less common (e.g., Executive Order 14031).

Following the passage, implementation outcomes have varied greatly. Some locales have successfully implemented disaggregation over time, while others continue to face difficulty years later.
A common thread across these experiences includes the following:

- **Funding remains a core issue.** Across conversations with multiple locales, funding is a consistent theme—many pass data disaggregation but lack the resources needed to compel agencies to implement it.
- **Implementation takes longer than legislation specifies.** Even with generous timelines, implementation is significantly delayed due to myriad factors, including deprioritization of data disaggregation and the reality that implementation is a distinct, under-resourced stage.
- **Including implementation details in legislation varies by locale.** Some locales weigh political feasibility and omit specific implementation language if it might hinder passage. Chicago's ordinance reflects this logic: separate the implementation process from the legislative process to secure passage first, then address implementation later.
- **Greater transparency from government agencies is a persistent need.** In locales such as New York State and Massachusetts, community organizations are not as involved in the implementation process as they anticipated following legislative passage.[96] [97] For their part, community advocacy organizations describe their post-passage role as one of "nudging" legislators, rather than actively participating in the rollout of the legislation.[98]
- **Weakened political will for data disaggregation is a persistent challenge.** In locales such as Colorado, state governments have acknowledged that they are unable to bring data disaggregation forward to state officials as they would like

[96] Savita, interview.
[97] Lloyd Feng (Senior Data Policy Coordinator, CACF) interview with the author, March 2026.
[98] Savita, interview.

due to federal anti-DEI initiatives.[99] For their part, state officials describe their role as one of hesitant advocacy, citing possible repercussions from the federal government and potential lawsuits, rather than actively championing data disaggregation efforts.

The following recommendations address the key implementation gaps identified above.

Recommendations

Use Language as a Disaggregation Proxy

Encourage localities to include language as a disaggregating variable. Language data can serve as a useful proxy for analyzing socioeconomic differences, immigration trends, and further community disaggregation. In addition, collecting this information creates secondary benefits: it allows for more targeted policy interventions—for example, if a community speaks a specific dialect of Lao, medical facilities can better plan translation services.

An example that illustrates the importance of language disaggregation would be within the Chinese American community. Many U.S. cities are home to large Chinese American populations, which vary in linguistic background and cultural practices by region and by when they immigrated to the U.S. Immigration waves have shaped socioeconomic outcomes, and language can help track those differences. Earlier Chinese immigrants, many of whom arrived decades ago, tend to speak Cantonese or other non-Mandarin dialects, while more recent immigrants are more likely to speak Mandarin.[100] These groups differ not only in language but also in educational backgrounds, economic opportunities, and settlement patterns. Current census categories do not capture this variation, but collecting data on language spoken at home would provide a much clearer picture of migration trends and allow for more nuanced analysis. Collecting language data also yields a positive externality: it can aid government outreach and targeted campaigns. Readily available information on the languages citizens speak enables local governments to determine which language services to provide in healthcare, education, and other sectors. Localities such as New

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[99] Tsuchiya, interview.

[100] Zheng-sheng Zhang, "The Chinese Diaspora (Chapter 19) - Chinese Signs," Cambridge Core, February 29, 2024, https://www.cambridge.org/core/books/abs/chinese-signs/chinese-diaspora/50061CB3E2B4EDA2FD0E6C01BAF1A4AD.

York City, Portland, and California have all passed data disaggregation legislation that includes disaggregation by language spoken.[101] [102] [103]

Incorporating language as a disaggregation variable would provide policymakers sharper analytical tools and enable more responsive services. It is a practical step that builds on existing efforts to disaggregate racial and ethnic data and moves us closer to understanding the true diversity within our communities. While implementation may be difficult in certain localities and across some government agencies, the potential returns for communities that can incorporate language disaggregation (even within specific agencies like family or health services) could be transformative.

Implementation

I recommend that legislation on data disaggregation include language requiring localities to report back on their implementation process and involve community organizations in the implementation process.

One consistent observation over the past six years is that data disaggregation takes time, and unexpected obstacles arise. While a possible solution is to encourage legislators to incorporate fixed timelines or set dates for implementation, this may weaken political will. Legislators are often reluctant to support bills that constrain them, especially when deadlines conflict with other priorities. Thus, specific timelines can become a hindrance to passage.

A more helpful approach is to require state agencies to release public updates on their progress with data disaggregation and to create forums for community advocacy organizations to share their insights. In locales such as Massachusetts and New York, community organizations have expressed concern that, once legislation was passed, they were not as involved in implementation as they had hoped. [104] [105]

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[101] California Assembly Bill 1358, Reg. Sess. (2021–2022), amended in Senate July 8, 2021, https://leginfo.legislature.ca.gov/faces/billTextClient.xhtml.
[102] New York City Local Law No. 126, "A Local Law to amend the New York city charter, in relation to the collection of demographic data regarding ancestry and languages spoken" (2016), https://nyc.legistar1.com/nyc/attachments/f959cc8b-4700-437b-b1a7-675805361cc4.pdf.
[103] City of Portland, Office of Equity, "ADM-18.03: Race, Ethnicity, Language, Disability, and Tribal Affiliation Demographic Data Standards," adopted May 2, 2024, https://www.portland.gov/policies/administrative/civil-rights/adm-1803-race-ethnicity-language-disability-and-tribal.
[104] Feng, interview.
[105] Savita, interview.

Future data disaggregation policies should include provisions that:

1. **Mandate routine public updates:** Require state agencies to publish regular progress reports (e.g., monthly) detailing their data disaggregation efforts. For example, a state department of health would post an update explaining where they are in the process, what barriers exist, and what steps are next.
2. **Establish community forums:** Create formal channels for community advocacy organizations serving impacted populations to consult with legislators and agencies during the rollout phase.

These two provisions would increase transparency and ensure community involvement in implementation. City and state legislators manage many competing priorities; community advocacy groups often have strong relationships with legislators and deeper insight into the communities most affected. The goal is not to burden lawmakers but to provide community insight that helps government agencies implement legislation effectively.

Data Privacy and Security Safeguards

Recent court rulings have permitted U.S. Immigration and Customs Enforcement (ICE) to access Medicaid enrollee data, including addresses, phone numbers, and citizenship status, for deportation purposes.[106] As of January 2026, ICE can access this data without restriction in 28 states, and even in the 22 states that successfully sued for limited protections, basic identifying information remains accessible.[107]

The current political climate around immigration has direct implications for AANHPI communities. Approximately 65% of the AANHPI population are foreign-born, making them disproportionately susceptible to detention and deportation proceedings.[108] Moreover, a recent Supreme Court decision cleared the way for ICE agents to consider race, language, accent, and employment location when conducting immigration stops.[109] This has fueled fears of racial profiling and further erodes trust in systems that collect information around race and ethnicity. Without proactive safeguards, the very data intended to reveal disparities and direct resources could become a tool for surveillance and harm.

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[106] Nicole Acevedo, Laura Strickler , and Alicia Victoria Lozano, “ICE Will Get Access to Medicaid Enrollees’ Personal Information to Help Find Immigrants.,” NBCNews.com, July 17, 2025, https://www.nbcnews.com/news/latino/ice-gets-access-medicaid-personal-data-rcna219462.

[107] Ali Swenson, “After Judge’s Ruling, HHS Authorized to Resume Sharing Some Medicaid Data with Deportation Officers,” AP News, January 5, 2026, https://apnews.com/article/medicaid-data-hhs-rfk-sharing-immigration-trump-30784ce01a403a16aca504980e4c7bc9.

[108] Karthick Ramakrishnan, Connie Tan, Akil Vohra, and Lisa Ocampo, “By the Numbers: Immigration. AAPI Data Guide on Timely Policy Issues” (Berkeley, CA: AAPI Data, 2025).

[109] Noem v. Vasquez Perdomo, 606 U.S. ___ (2025).

Accordingly, I recommend the following protection mechanisms:

1. **Data Suppression:** When compiling information related to race and ethnicity, cities and/or states should be encouraged to omit unnecessary identifiers that could compromise individual privacy, including names, addresses (beyond ZIP code or county), Social Security numbers, and other personal identifiers. Suppression does not eliminate analytic value but reduces identification risk.
2. **Data Masking (Encryption):** Localities and partner organizations should encrypt all demographic data. Decryption keys should be held only by state or local entities, and access should be logged and audited regularly.
3. **Sample Size Thresholds (Small Cell Suppression):** If an ethnic group falls below a predetermined sample size within a given geographic unit (e.g., zip code or county), localities should not report that group separately. Instead, those individuals should be placed under a general “Other” category. Not to aggregate those individuals into larger groups in a way that erases their identity, but to protect them from identification. For example, if there are only ten Marshallese individuals in Cook County, Illinois, even suppressed data (e.g., “Marshallese: <10”) could enable re-identification. Small cell suppression is an established statistical control method that directly addresses this vulnerability.
4. **Multi-Stage Access Control for Third-Party Organizations:** For nonprofit and health organizations collecting disaggregated demographic data directly (e.g., via community health surveys or social service intake), access to that data should be gated behind multiple, auditable barriers. Modeled on the precedent set by the Lotus Project,[111] this process would require any individual or entity seeking access to complete the following requirements before viewing data:
    a. Submit a formal Data Use Agreement specifying the intended purpose of the data, data security protocols, expected outputs, etc.
    b. Complete a questionnaire detailing their institutional affiliation, whether they have ever shared data with federal immigration enforcement, how they will store and dispose of data, and what they intend to produce from the analysis
    c. Receive approval from a designated data steward or community review committee, which has the authority to deny access for any purpose deemed harmful, including immigration enforcement.

    Each step creates an independent barrier. Even if a bad actor misrepresents themselves on the agreement, the questionnaire and approval process provide additional opportunities to identify and block nefarious intent. This layered approach ensures that data is accessible only to legitimate researchers, advocates, and service providers, not to entities seeking to use disaggregated information for detention or deportation.

Taken together, these recommendations create a layered data protection framework that preserves the benefits of disaggregation—revealing hidden disparities and directing resources to underserved AANHPI subgroups—while mitigating the very real risks posed by federal surveillance and immigration enforcement.

Coalition Building

Organizations in localities that built strong coalitions were successful not only in advancing data disaggregation efforts for AANHPI communities but also for other communities in the locale. Successful examples include New York State and California. In both cases, AANHPI advocacy organizations built broad coalitions, both across different sectors within the AANHPI community and with other coalitions representing communities that would also benefit from data disaggregation. For instance, in California in 2021, legislation (AB 1358) was introduced that would have advanced data disaggregation for AANHPI, Latine, MENA, Black, and other groups as well.[110] As a result, the legislation had a broad base of support from AANHPI organizations but also Black and Latine organizations across California. The legislation unfortunately did not pass, but there is power in building broad coalitions with different groups to advance these data disaggregation efforts.

Additionally, when interviewing community organizations in Massachusetts about their data disaggregation efforts, they stated that they wished they had built more coalitions with other Black, Indigenous, and People of Color (BIPOC) communities in the area and that they had engaged in political education within the AANHPI community of Massachusetts prior to advancing legislation.[111] Furthermore, once one community advances data disaggregation, other communities can more easily implement their data disaggregation efforts. For instance, in New York State, data disaggregation was passed for AANHPI communities, and eventually there was success in advancing data disaggregation for Black and MENA communities as well.

Therefore, localities advancing data disaggregation efforts should build coalitions with non-AANHPI organizations and engage in proactive political education within the AANHPI community. There are mutual long-term benefits to advancing data disaggregation for all communities in a way that reflects each locale's unique demographics.

[110] Sign this petition for AB 1358: Data Disaggregation & Health Equity," Action Network, accessed April 10, 2026, https://actionnetwork.org/petitions/ab-1358-ca.
[111] Savita, interview.

Conclusion

In conclusion, disaggregated data collection is not merely an adjustment to demographic reporting but a necessary step towards equitable governance. The experiences of states and localities such as Colorado, Chicago, Massachusetts, Oregon, and Portland demonstrate the promise and the complexity of implementing data disaggregation. While their approaches may differ, the evidence consistently shows that granular data improves the ability of governments and community organizations to identify disparities, allocate resources more equitably, and design policy interventions. At the same time, these case studies also make clear that implementation cannot occur efficiently without political will, adequate funding, and meaningful community engagement. Data disaggregation is more effective when paired with a transparent implementation process, regular public reporting, and strong partnerships between government agencies and community organizations. Additionally, growing concerns around immigration enforcement and federal access to sensitive information underscore the need for robust privacy protections. Without these protections, communities may lose trust in the same systems designed to support them. While the census will continue to serve as a national baseline, state and local governments are better equipped to collect timely, community-specific data that reflects the realities of rapidly changing demographics. By investing in disaggregation, policymakers can create a framework that recognizes the diversity within AANHPI communities and advances more equitable outcomes for all populations.